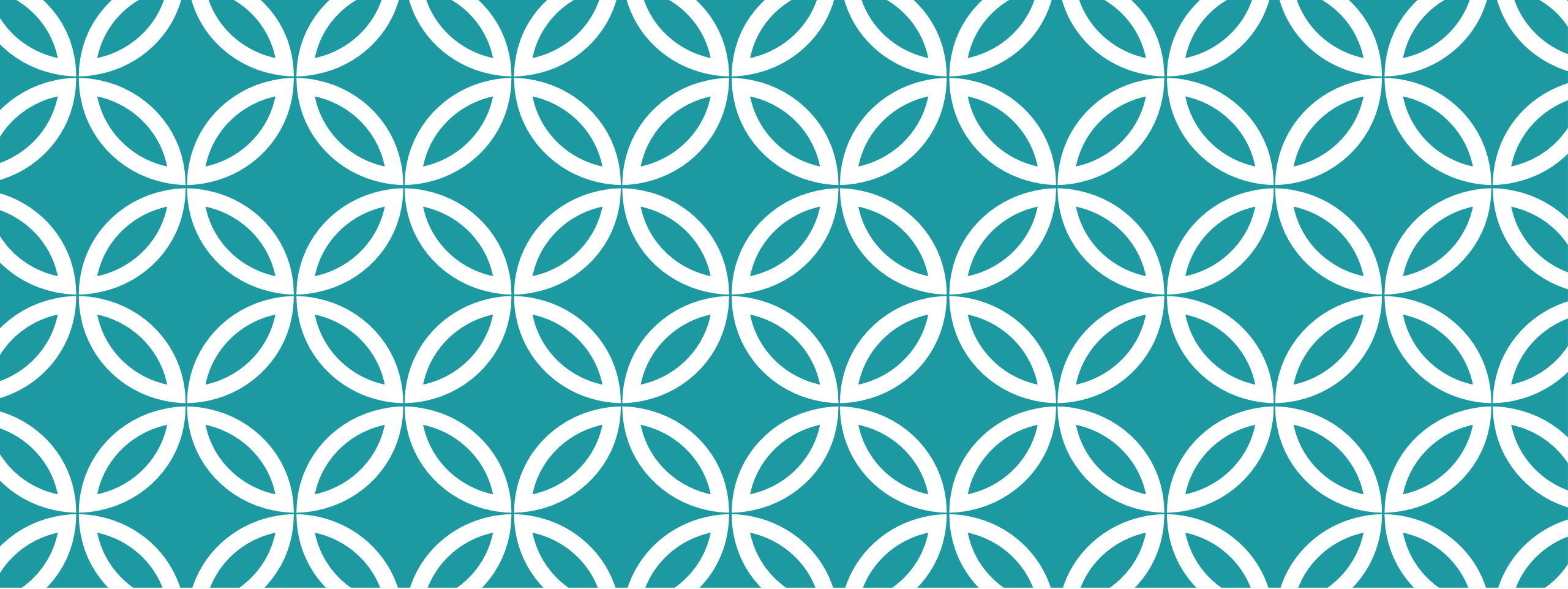


PUBP 233
INTRODUCTION TO DEMOGRAPHY
(POPULATION AND SOCIETY)

**Week 9: Proximate and Social
Determinants of Fertility**

Shivakumar Jolad



SOCIAL AND BIOLOGICAL DETERMINANTS OF FERTILITY



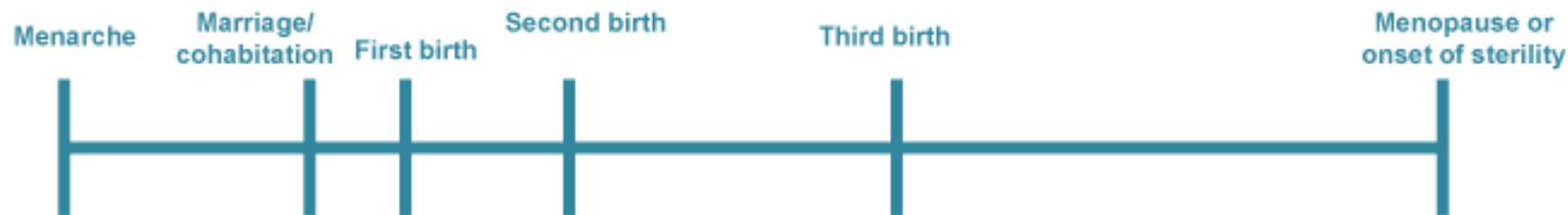
DETERMINANTS OF FERTILITY

Proximate, Immediate and distal factors influencing fertility

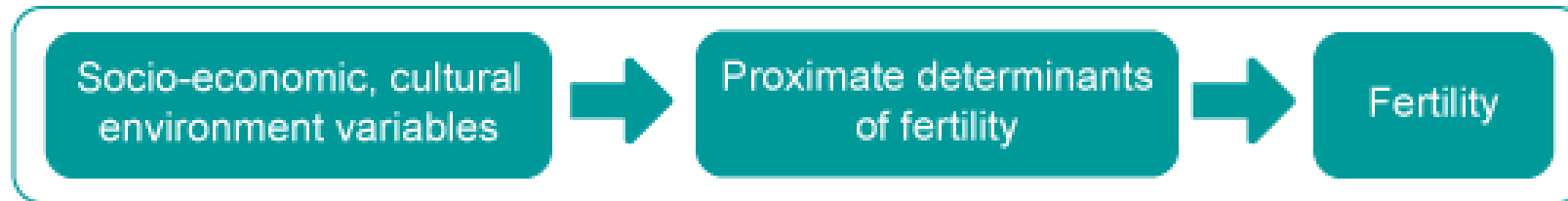
- Biological, behavioural, and social

Demographic Fertility versus Clinical Fertility

- Live births- add to population
- Clinical- potential of a woman to become pregnant and bear a live



CONCEPT OF PROXIMATE DETERMINANTS



The **proximate determinants of fertility** are the biological and behavioural factors through which social, economic, cultural and environmental variables influence fertility. They have direct effects on fertility.

Contraceptive use and sexual activity may be the most obvious factors affecting the level of fertility in different populations.

Many other factors also affect fertility levels.

TERMS

Fertility: Number of live birth which women give birth to. Actual reproduction. A woman is considered *fertile* if she is bearing live offspring.

Fecundity: The ability to reproduce, which refers both **the ability to conceive and the ability to successfully complete a pregnancy**. A woman is considered *fecund* if she is capable of bearing a live birth, even if she has not given a birth yet.

Fecundable: The ability to conceive

Infertility/Sterility: The inability to bear any children, either due to the inability to conceive or the inability to carry a pregnancy to a live birth.

Infecundity: The inability to have a live birth after several years of exposure to the risk of pregnancy.

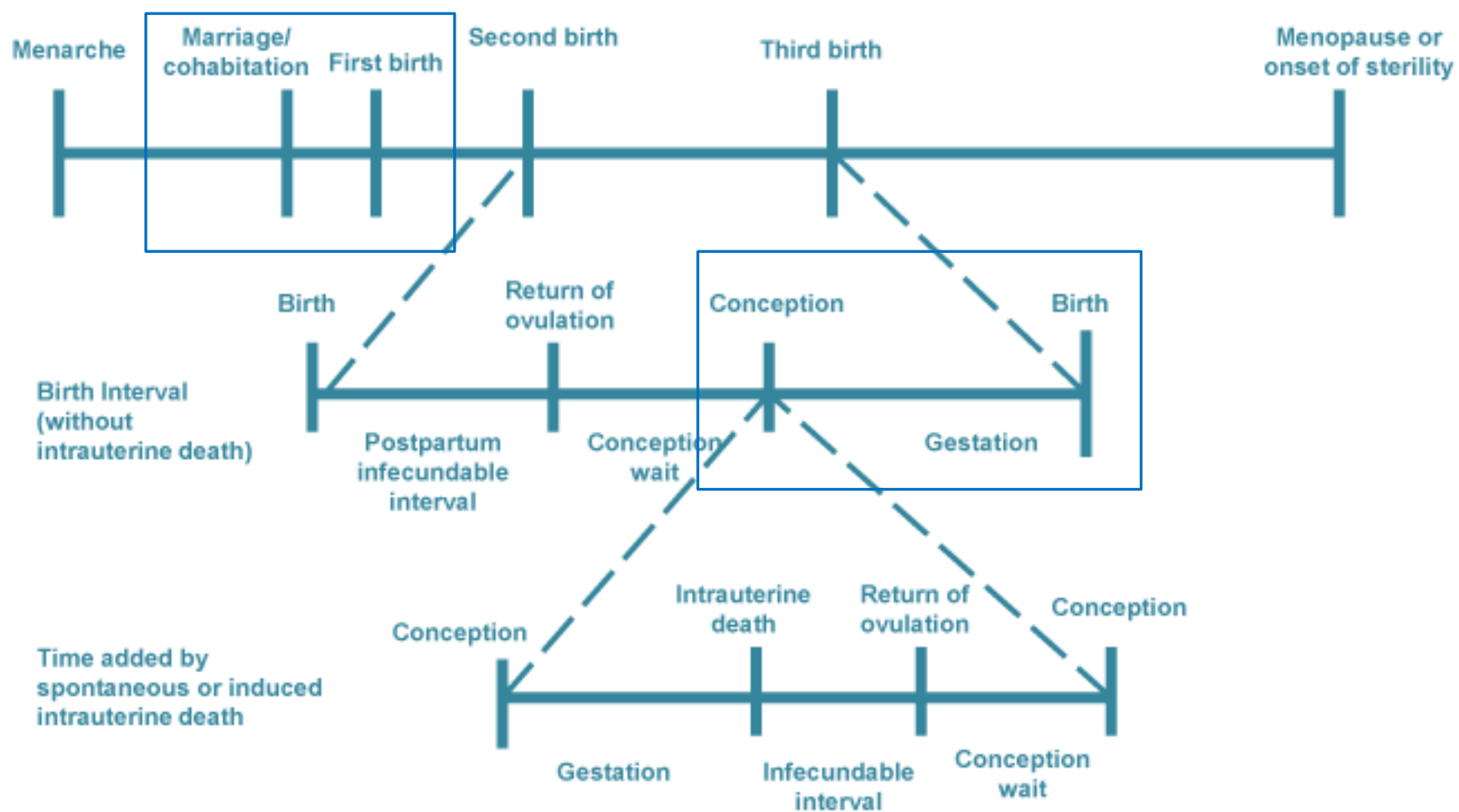
INTERMEDIATE/PROXIMAL DETERMINANTS

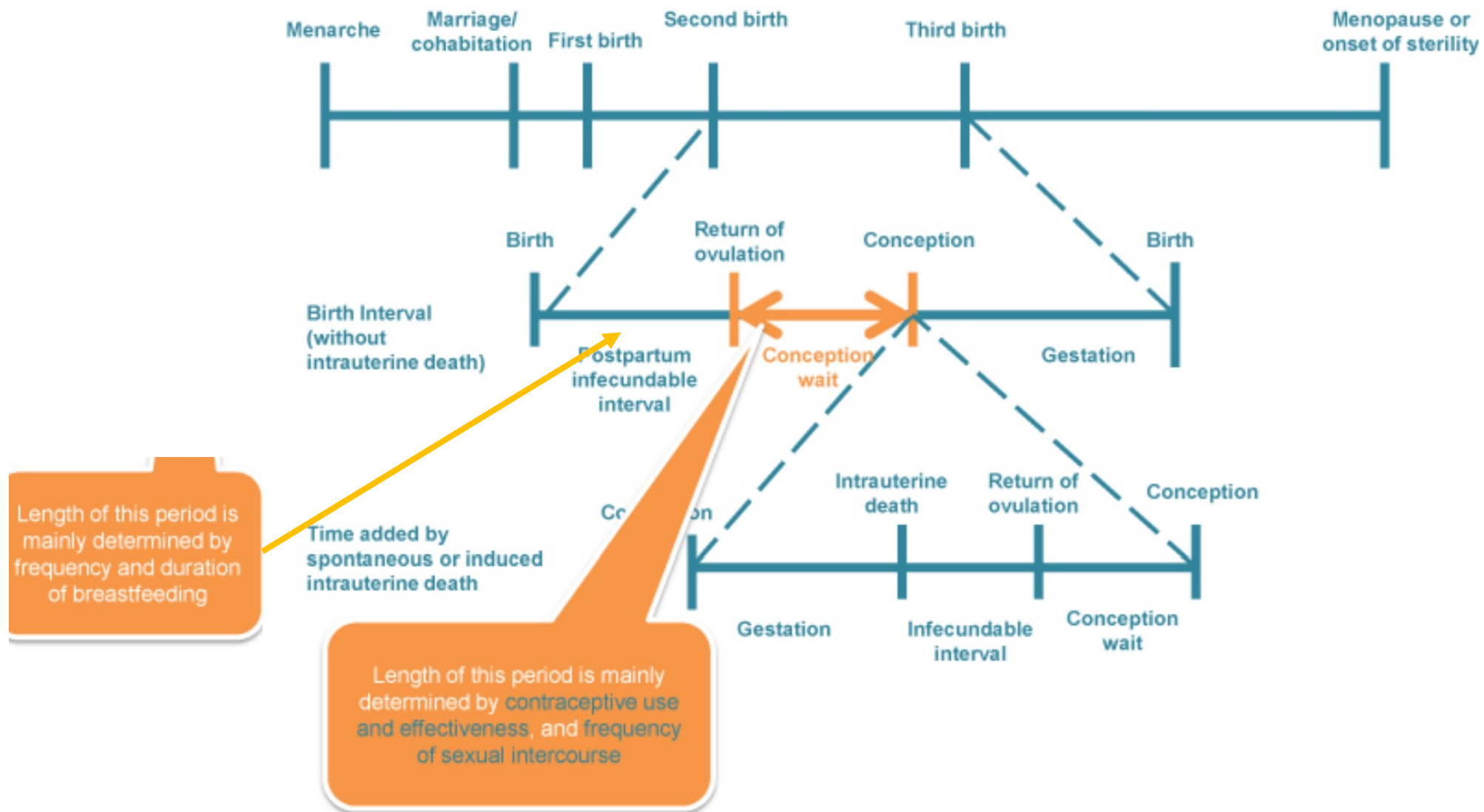
David and Blake 1956 (11 variable)

Factors affecting exposure to intercourse ('intercourse variables')

Factors affecting exposure to conception ('conception variables')

Factors affecting gestation and successful parturition ('gestation variables')



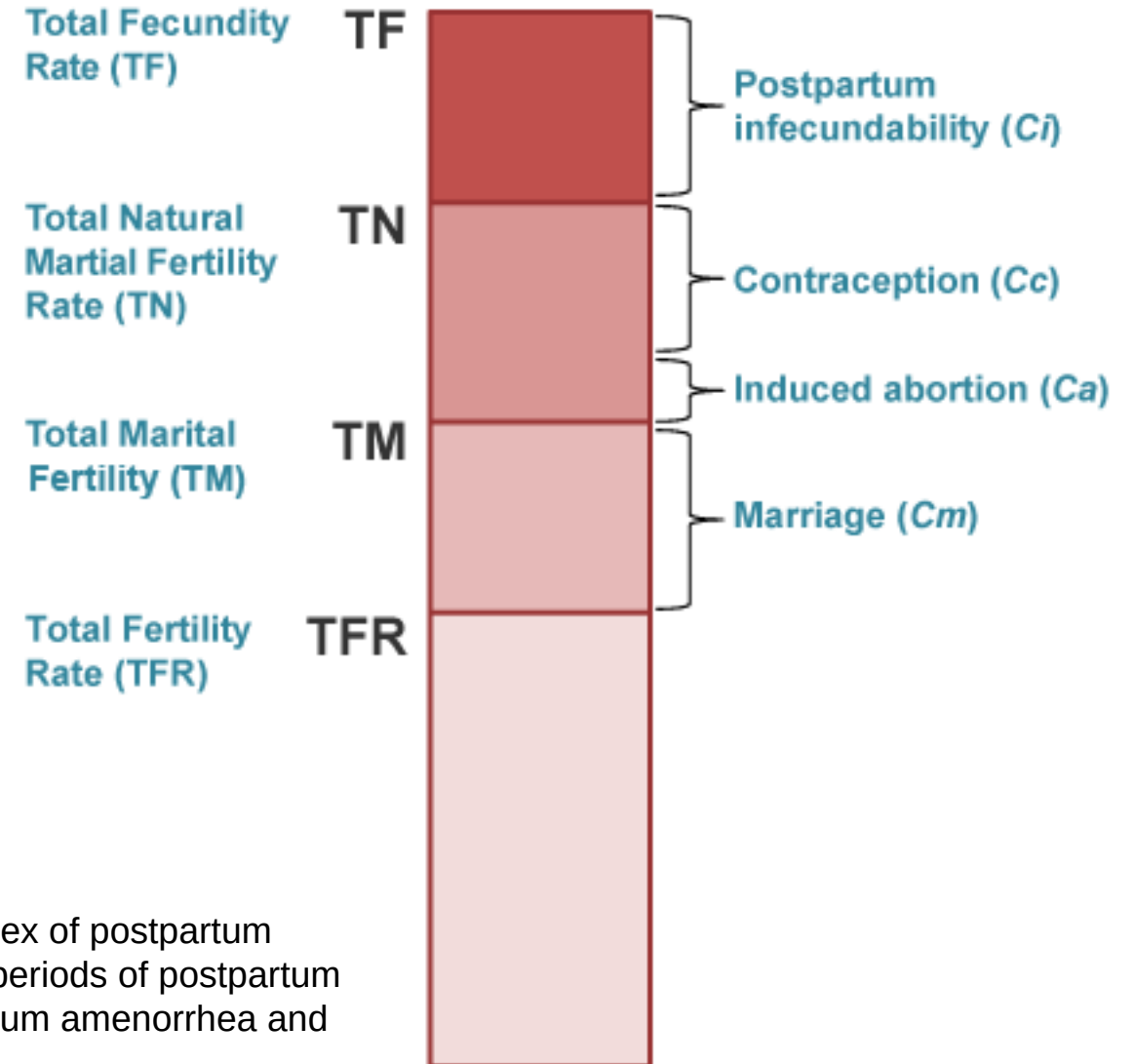


BONGARTS ACCELERATE MODEL

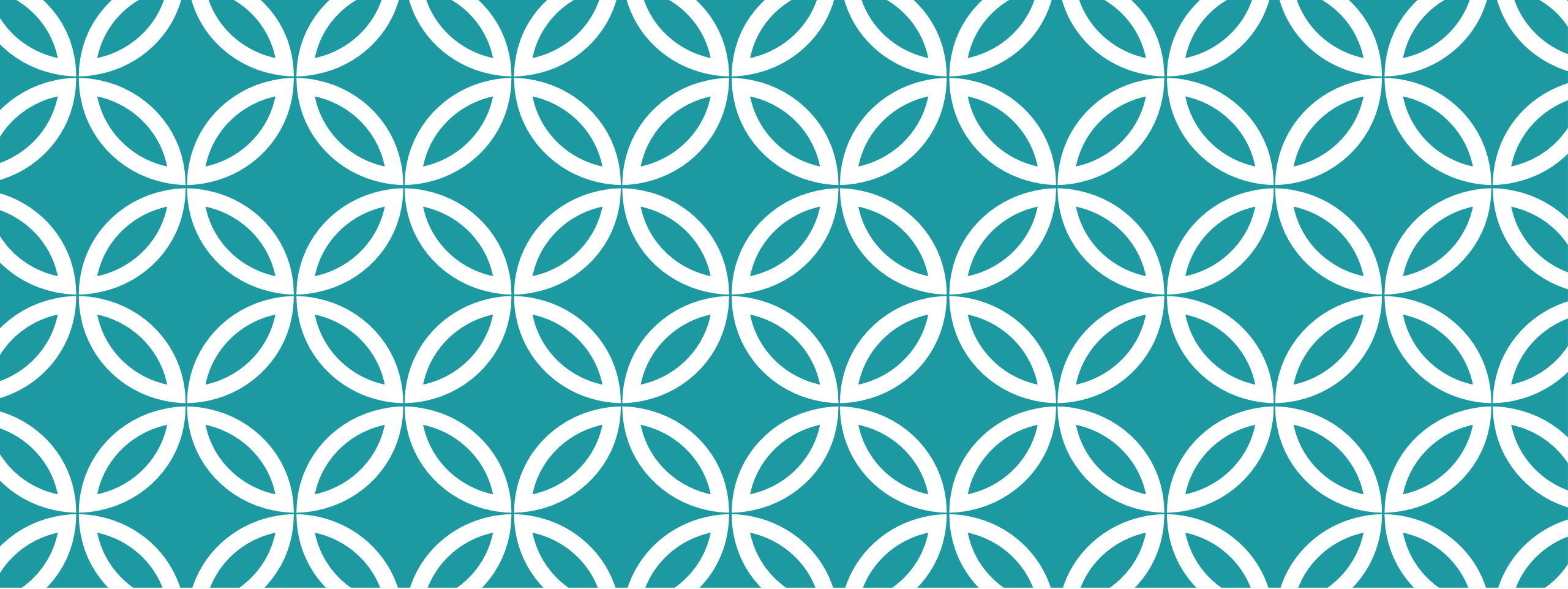
$$TFR = TF \times C_m \times C_i \times C_a \times C_c$$

where:

TFR = total fertility rate
 TF = total fecundity rate
 C_m = index of marriage
 C_i = index of postpartum infecundability
 C_a = index of abortion
 C_c = index of contraception

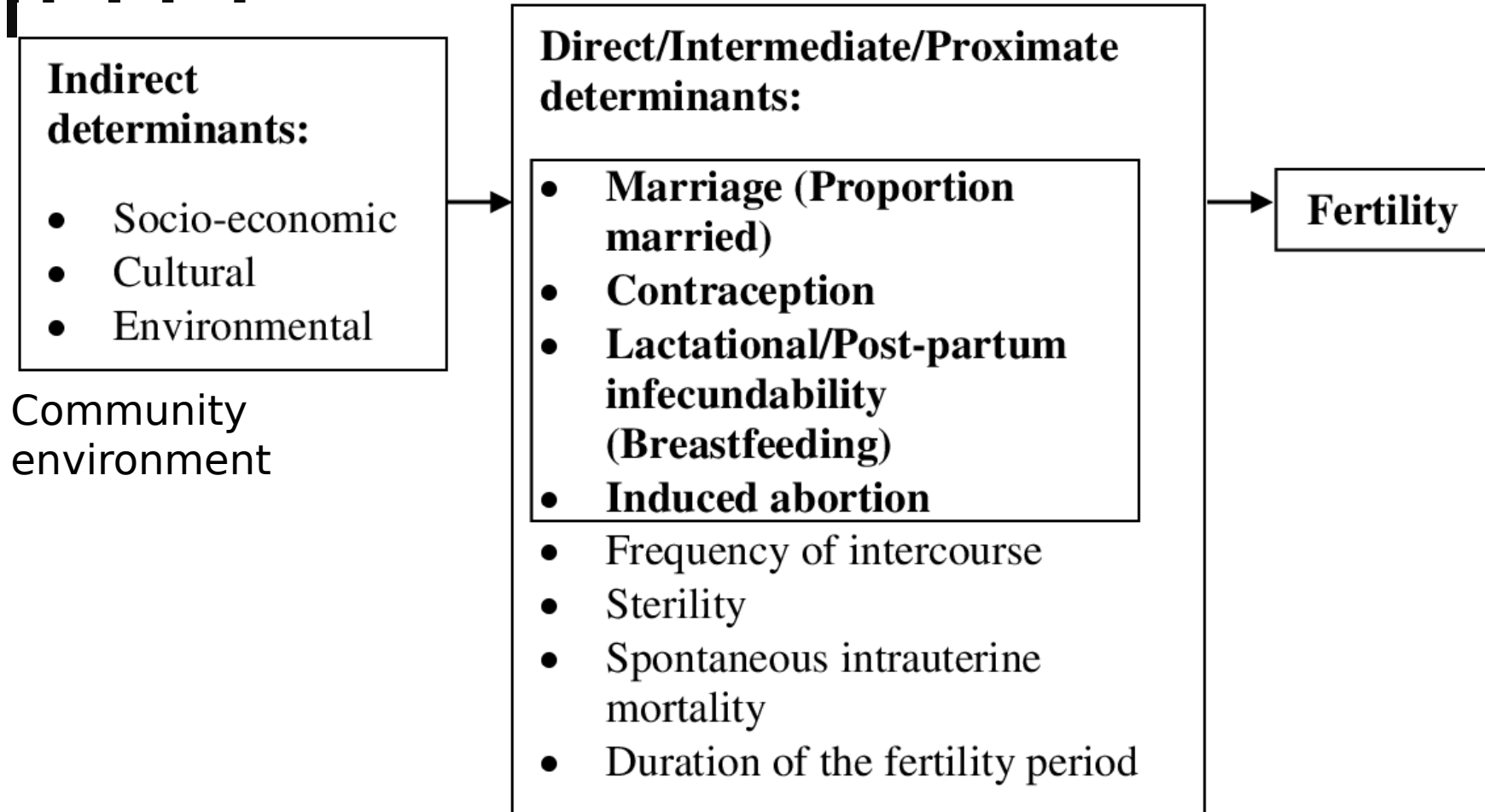


Postpartum infecundability (postpartum insusceptibility) -The index of postpartum infecundability is intended to measure the effects on fertility of extended periods of postpartum infecundability, or often called postpartum insusceptibility, due to postpartum amenorrhea and postpartum abstinenc



DISTANT DETERMINANTS OF FERTILITY

DISTANT DETERMINANTS OF FERTILITY



Recent fertility and its proximate determinants in 3 ASEAN countries / Lai Siow Li [Siow-Li Lai](#)

SOCIAL DETERMINANTS OF FERTILITY

Socioeconomic development has a decisive effect in lowering fertility in the long run.

Household factors:

- ▢ **Higher income, for instance, allows households to afford more children, but nevertheless often reduces fertility** because of its links to other factors like Education.
- ▢ **Education**, especially for women, **fairly reliably reduces fertility**, though its effect may take years to appear.
- ▢ **Improved health and lower mortality** also contribute to lower fertility, through both biological and behavioural channels.
- ▢ **The** **Reducing Fertility in Developing Countries** **contrast, is uncertain and**

A Review of Determinants and Policy Levers

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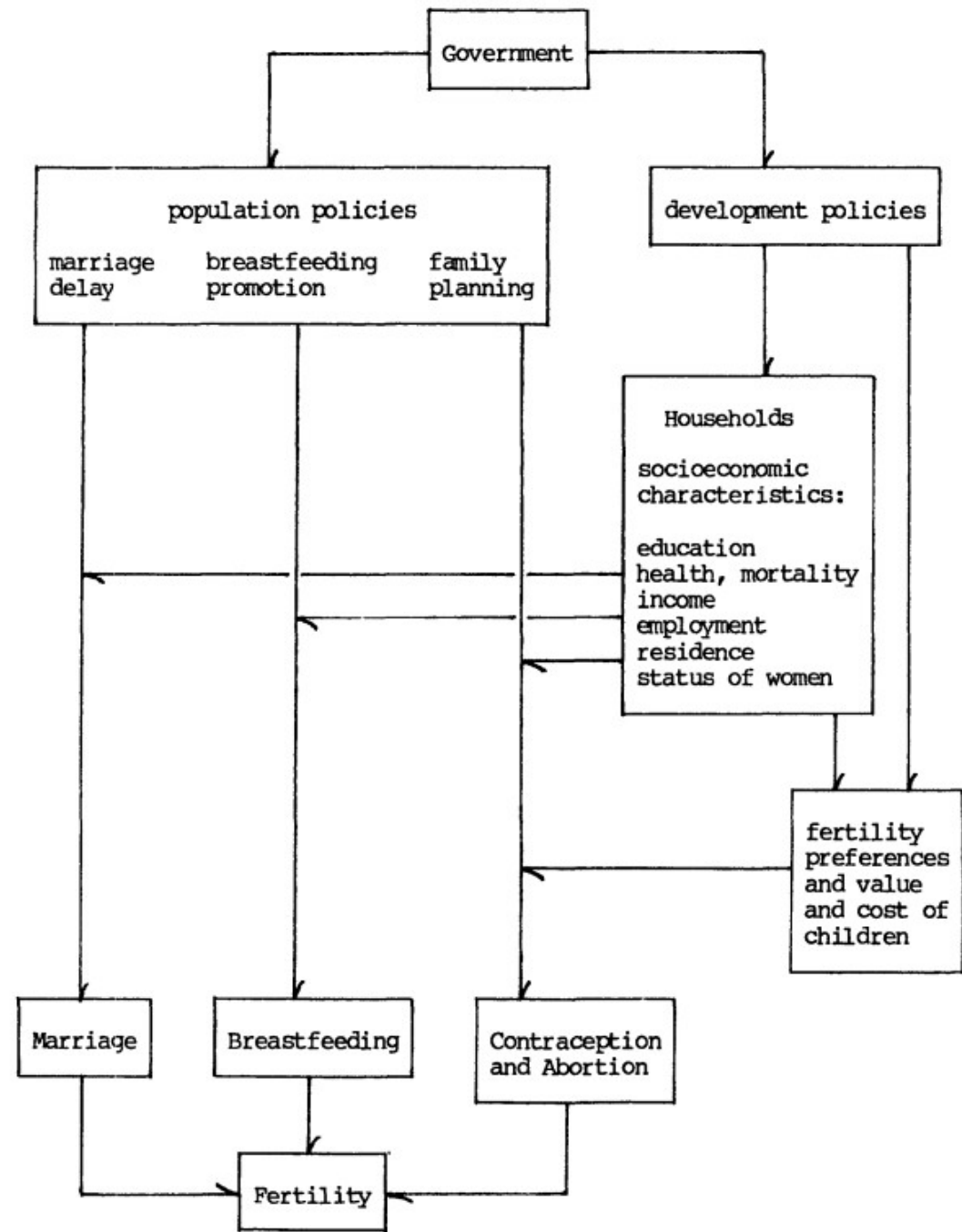
Rodolfo A. Bulatao

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POPULATION AND DEVELOPMENT SERIES
Number 5

GOVERNMENT INFLUENCE ON FERTILITY



NFHS- QUESTIONS FERTILITY AND ITS DETERMINANTS

CHAPTER 3 CHARACTERISTICS OF RESPONDENTS

- 3.1 Basic Characteristics of Survey Respondents
- 3.2 Schooling and Literacy
- 3.3 Mass Media Exposure
- 3.4 Employment
- 3.5 Occupation

CHAPTER 4 FERTILITY AND FERTILITY PREFERENCES

- 4.1 Current Fertility
- 4.2 Children Ever Born and Living
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- 4.7 Bathing Practices During Menstrual Period
- 4.8 Teenage Childbearing
- 4.9 Desire for Another Child
- 4.10 Ideal Family Size
- 4.11 Fertility Planning Status
- 4.12 Wanted Fertility Rates

CHAPTER 5 FAMILY PLANNING

- 5.1 Contraceptive Knowledge and Use
- 5.2 Source of Modern Contraceptive Methods
- 5.3 Informed Choice
- 5.4 Discontinuation of Contraceptives
- 5.5 Exposure to Family Planning Messages
- 5.6 Demand for Family Planning
- 5.7 Hysterectomy

CHAPTER 6 OTHER PROXIMATE DETERMINANTS OF FERTILITY

- 6.1 Marital Status
- 6.2 Age at First Marriage
- 6.3 Consanguineous Marriages
- 6.4 Age at First Sexual Intercourse
- 6.5 Recent Sexual Activity
- 6.6 Insusceptibility to Pregnancy
- 6.7 Pregnancy Outcomes
- 6.7.1 Characteristics of Abortions

CONCEPTION AND GESTATION VARIABLES

Conception

Fecundity or infecundity, as affected by involuntary causes

Use or non-use of contraception

Fecundity or infecundity, as affected by voluntary causes

Gestation

Foetal mortality from involuntary causes

Foetal mortality from voluntary causes

PROXIMATE AND DISTANT FACTORS



Contraceptive use and effectiveness



Frequency of sexual intercourse



Duration of postpartum amenorrhea

Proximate
determinants
of fertility



Duration of postpartum infecundability



Sterility



Marriage or being in sexual union



Induced abortion



Couple's income



Religion



Women's education

Distal
determinants
of fertility



Residence



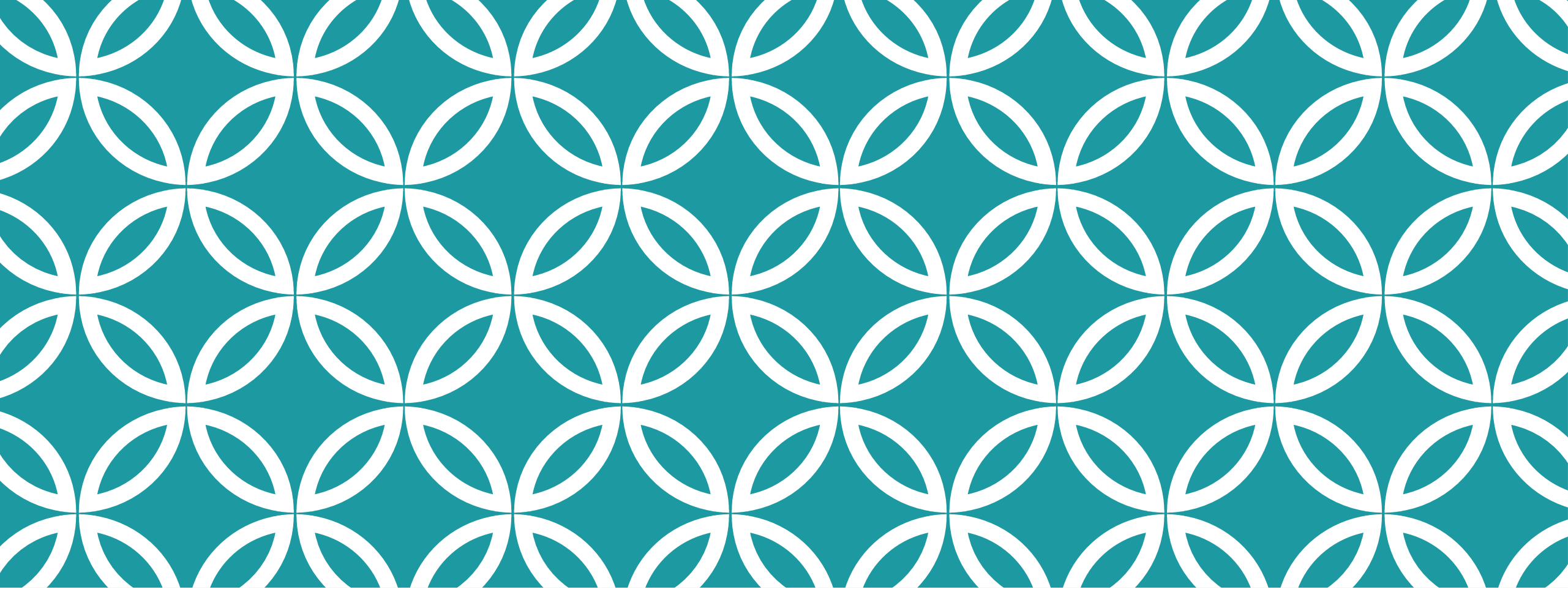
Governments population policies



Availability of child care



Access to contraceptives



DISTANT DETERMINANTS OF FERTILITY

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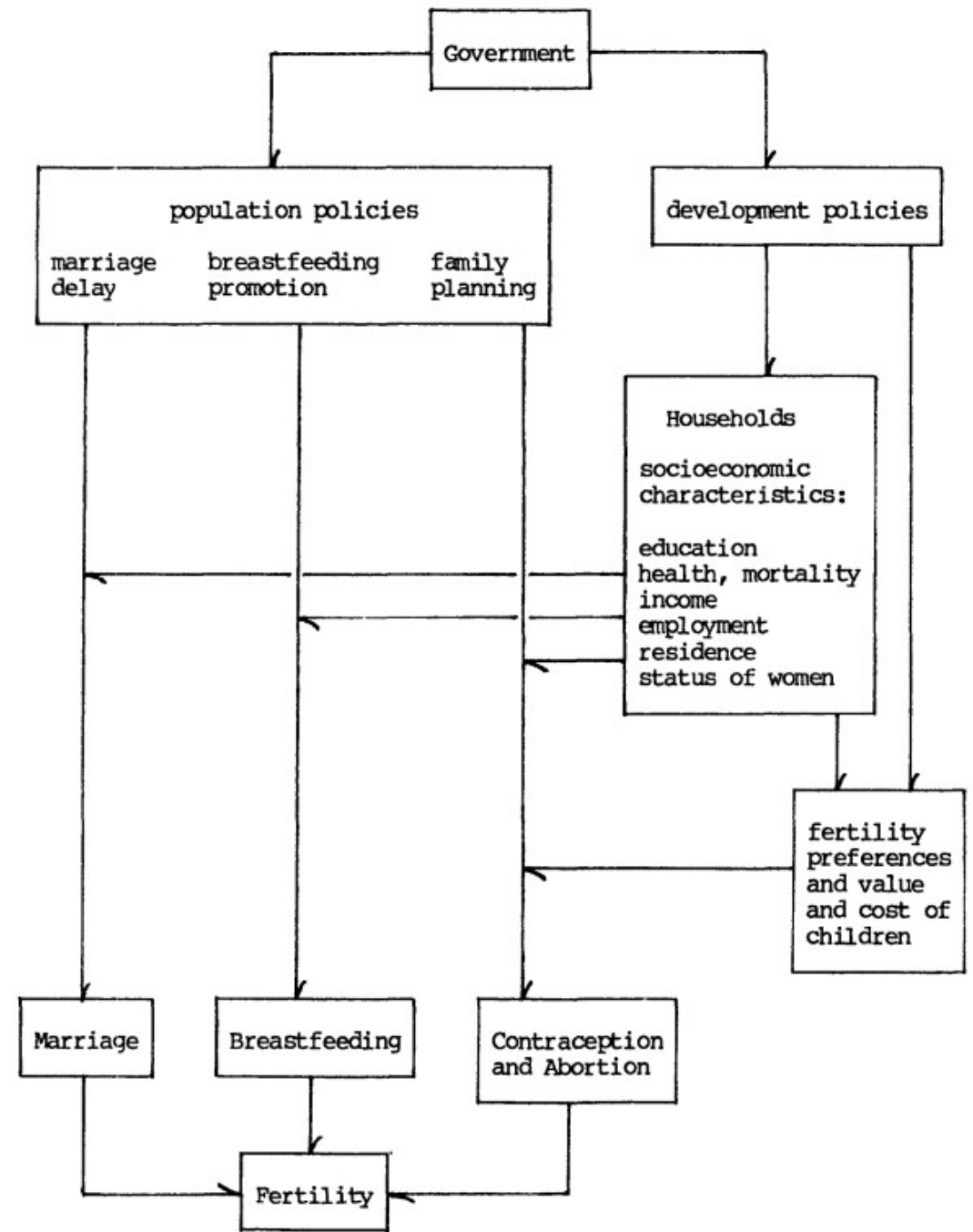
Rodolfo A. Bulatao

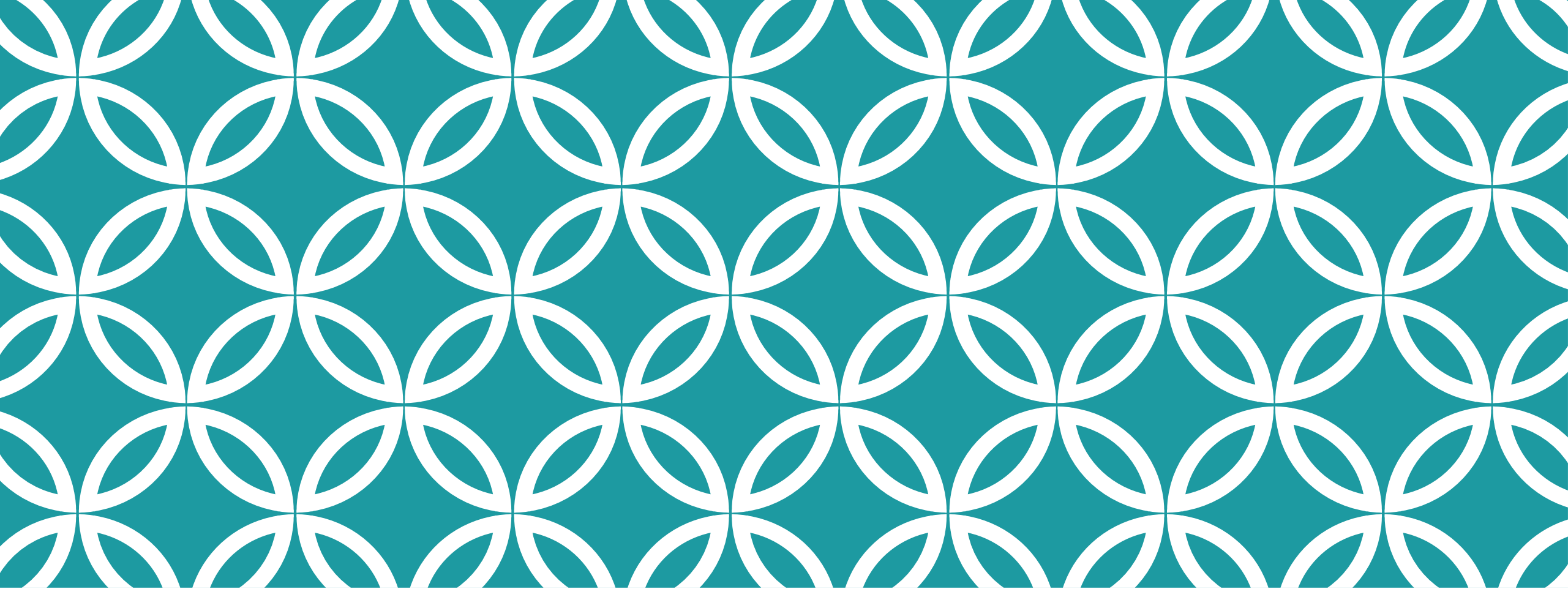
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NATIONAL FAMILY HEALTH SURVEY |

NATIONAL FAMILY HEALTH SURVEY (NFHS - 5), 2019–21



NFHS provides data on various indicators including population dynamics, family planning, child and maternal health, nutrition, adult health, and domestic violence.

The first phase of the fifth round of the National Family Health Survey (NFHS-5) was carried out in 2019-20, with its findings released in December 2020. The previous, fourth round of NFHS took place in 2015-16, marking a five-year gap between the two surveys.

Phase one of NFHS-5 disclosed data for 22 states and union territories, specifically covering 17 states and 5 union territories.

A total of 2,81,429 households were surveyed, involving interviews with 3,07,422 women and 43,945 men across the 17 states.

The survey's findings for these 17 states were focused on indicators related to population, health and nutrition, access to infrastructure, and gender equality.



CHAPTERS- NATIONAL REPORT

CHAPTER 1 INTRODUCTION

1.1	Survey Objectives
1.2	Sample Design
1.3	Questionnaires
1.4	Biomarker Measurements and Tests
1.5	Pretest
1.6	Training of Field Staff
1.7	Fieldwork
1.8	Strategy to Ensure Data Quality
1.9	Data Processing
1.10	Response Rates

CHAPTER 2 HOUSEHOLD POPULATION AND HOUSING CHARACTERISTICS

2.1	Drinking Water Sources and Treatment
2.2	Sanitation
2.3	Exposure to Smoke inside the Home and Other Housing Characteristics
2.3.1	Exposure to Smoke inside the Home
2.3.2	Other Housing Characteristics
2.4	Household Wealth
2.5	Hand Washing
2.6	Household Population and Composition
2.7	Birth Registration
2.8	Death Registration
2.9	Children's Living Arrangements and Parental Survival
2.10	Schooling
2.10.1	Educational Attainment
2.10.2	Preschool Attendance
2.10.3	School Attendance
2.11	Disability
2.12	Use of Tobacco and Alcohol
2.13	Possession of Mosquito Nets

FERTILITY

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- 6.7 Pregnancy Outcomes
- 6.7.1 Characteristics of Abortions

MORTALITY AND CHILD HEALTH, NUTRITION

CHAPTER 7 INFANT AND CHILD MORTALITY

- 7.1 Infant and Child Mortality.....
- 7.2 Biodemographic Risk Factors
- 7.3 Perinatal Mortality

CHAPTER 8 MATERNAL HEALTH.....

- 8.1 Pregnancy Registration
- 8.1.1 Registration of Pregnancies.....
- 8.1.2 Mother and Child Protection Card (MCP Card).....
- 8.2 Antenatal Care Coverage and Content
- 8.2.1 Skilled Providers
- 8.2.2 Timing and Number of ANC Visits
- 8.3 Components of ANC Visits.....
- 8.4 Protection against Neonatal Tetanus
- 8.5 Ultrasound Testing during Pregnancy.....
- 8.6 Delivery Services
- 8.6.1 Institutional Deliveries
- 8.6.2 Skilled Assistance during Delivery
- 8.6.3 Delivery by Caesarean Section
- 8.7 Delivery Costs.....
- 8.8 Postnatal Care
- 8.8.1 Postnatal Health Check for Mothers
- 8.8.2 Postnatal Health Checks for Newborns.....

CHAPTER 9 CHILD HEALTH.....

- 9.1 Birth Weight.....
- 9.2 Vaccination of Children
- 9.3 Symptoms of Acute Respiratory Infection
- 9.4 Fever
- 9.5 Diarrhoeal Disease
- 9.5.1 Prevalence of Diarrhoea
- 9.5.2 Treatment of Diarrhoea
- 9.5.3 Feeding Practices
- 9.5.4 Knowledge of ORS Packets
- 9.6 Disposal of Children's Stools.....
- 9.7 Utilization of Integrated Child Development Services (ICDS).....
- 9.7.1 Utilization of ICDS by Pregnant and Lactating Mothers

CHAPTER 10 NUTRITION AND ANAEMIA.....

- 10.1 Nutritional Status of Children
- 10.1.1 Nutritional Status among Young Children
- 10.1.2 Levels of Child Malnutrition.....
- 10.2 Infant and Young Child Feeding Practices.....
- 10.2.1 Initiation of Breastfeeding.....
- 10.2.2 Exclusive Breastfeeding.....
- 10.2.3 Median Duration of Breastfeeding
- 10.2.4 Complementary Feeding
- 10.2.5 Minimum Acceptable Diet
- 10.3 Anaemia Prevalence in Children.....
- 10.4 Presence of Iodised Salt in Households
- 10.5 Micronutrient Intake and Supplementation among Children
- 10.6 Nutritional Status in Adults.....
- 10.7 Waist-to-Hip Ratio in Adults
- 10.8 Anaemia Prevalence in Adults
- 10.9 Food Consumption of Women and Men

MORBIDITY, HEALTH CARE, HIV

CHAPTER 11 MORBIDITY AND HEALTH CARE

11.1	Tuberculosis
11.1.1	Prevalence of Tuberculosis
11.1.2	Knowledge and Attitudes toward Tuberculosis
11.2	Health Problems
11.3	Use of Tobacco
11.3.1	Consumption of Tobacco
11.3.2	Quitting Tobacco.....
11.4	Alcohol Use, Health Insurance, and Sources of Health Care
11.4.1	Use of Alcohol
11.5	Health Insurance Coverage
11.6	Sources of Health Care.....
11.7	Reasons for Not Using Government Health Care
11.8	Recent Contact with Health Workers
11.9	Problems in Accessing Health Care

CHAPTER 12 OTHER ADULT HEALTH ISSUES

12.1	Coverage of Testing for Blood Pressure and Random Blood Glucose Measurements
12.2	Blood Pressure
12.2.1	Self Reports of Blood Pressure Measurement and Medication
12.2.2	Blood Pressure Levels and Treatment Status
12.3	Random Blood Glucose
12.4	Health Examinations for Cancer Screening
12.5	Age-Specific Death Rates and Crude Death Rates.....
12.6	Adult Mortality.....

CHAPTER 13 HIV/AIDS-RELATED KNOWLEDGE, ATTITUDES, AND BEHAVIOUR

13.1	HIV/AIDS Knowledge, Transmission, and Prevention Methods.....
13.2	Comprehensive Knowledge
13.3	Knowledge about Mother-to-Child Transmission.....
13.4	Accepting Attitudes toward People Living with HIV
13.5	Attitudes toward Negotiating Sex with Husband
13.6	Multiple Sexual Partners
13.7	Paid Sex.....
13.8	Coverage of HIV Testing Services.....
13.8.1	Awareness of HIV Testing Services and Experience with HIV Testing
13.8.2	HIV Testing of Pregnant Women.....
13.9	Self-Reporting of Sexually Transmitted Infections.....
13.10	HIV/AIDS-related Knowledge and Behaviour among Young People
13.10.1	Knowledge of HIV/AIDS.....
13.10.2	First Sex
13.10.3	Premarital Sex
13.10.4	Multiple Sexual Partners
13.10.5	Coverage of HIV Testing Services.....

WOMEN EMPOWERMENT; DOMESTIC VIOLENCE

CHAPTER 14 WOMEN'S EMPOWERMENT.....

14.1	Currently Married Women's and Men's Employment.....
14.2	Control over Women's Earnings.....
14.3	Control over Men's Earnings.....
14.4	Participation in Household Decision Making.....
14.5	Men's Attitudes toward Women's Roles in Decision Making.....
14.6	Women's Access to Money and Microcredit.....
14.7	Freedom of Movement.....
14.8	Attitudes toward Wife Beating.....
14.9	Attitudes toward Negotiating Safer Sexual Relations with Husband.....
14.10	Women's and Men's Ownership of Assets.....
14.11	Ownership and Use of a Mobile Phone.....

CHAPTER 15 DOMESTIC VIOLENCE.....

15.1	Measurement of Violence.....
15.2	Women's Experience of Physical Violence.....
15.2.1	Perpetrators of Physical Violence.....
15.3	Experience of Sexual Violence.....
15.3.1	Prevalence of Sexual Violence.....
15.3.2	Perpetrators of Sexual Violence.....
15.4	Experience of Different Types of Violence.....
15.5	Marital Control by Husband.....
15.6	Forms of Spousal Violence.....
16.6.1	Prevalence of Spousal Violence.....
15.7	Injuries to Women due to Spousal Violence.....
15.8	Violence Initiated by Women against Husbands.....
15.9	Help-seeking among Women who have Experienced Violence.....
16.9.1	Sources of Help.....

**CAIRO CONFERENCE
1994
ADVANCING
WOMEN'S RIGHTS**

Devika Makkat and Mrinalini
Godara



CAIRO CONFERENCE- 1994

“a new definition of population policy was advanced, **giving prominence to reproductive health and the empowerment** of women while downplaying the **demographic rationale for population policy**”

- The ICPD was the largest intergovernmental conference on population and development ever held,
- 179 governments participating and some 11,000 registered participants
- governments, UN specialized agencies and organizations, intergovernmental organizations, non-governmental organizations and the media.

NEW PARADIGM

By 1994: widespread consensus that:

- Population growth has a direct correlation with decreased standard of living
- Access to modern contraceptives needs to increase and population policies need to be adopted to decrease fertility

Cairo Document: **Individualistic >>populationist view**

Movements by feminist groups for better family planning, legalisation of abortion, access to contraceptives, etc.

SOME CRITICAL FEATURES

- Involvement of NGOs
- Environmental Concerns in the context of population
- Broadened the concept of family planning to include reproductive and sexual health
 - STDs
 - Unsafe Abortion
 - Maternal Mortality
- Reaffirmation of sovereign rights
- **Important actors:**
 - NGOs
 - Women's rights groups
 - US Government
 - The Holy See

NEW MODEL

Programs that are demographically driven, and are intended to act directly on fertility, are inherently coercive and abusive of women's right to choose the number and timing of their children.



"empower" women by increasing their educational levels, providing them with satisfying jobs, lightening their domestic responsibilities, and otherwise raising their status in the family and

PREPARATORY PROCESS

- Steps to avoid the overt politicisation of the Conference
- Individual vs Societal approach to population
- Abortion as Public health issue than women's rights

POLITICAL ENVIRONMENT

Cairo

- Fall of Soviet Union
 - Israel-Palestine Liberation Organization Peace agreement
 - Decline of US presence from International responsibilities
 - Japan's silence
 - Subdued G77
- Africa was “due” to have a major conference
 - Egypt: a moderate Muslim state
 - Egypt and the G77

NEW PARADIGM

- ❑ Assumption that fertility will not fall until a number of preconditions have been satisfied:
 - children survive
 - men take responsibility for contraception
 - women's right to control their own fertility
 - political power that will enable women to secure this right

- ❑ Feminist rhetoric over Family Planning

THE WOMEN'S MOVEMENT AT CAIRO

Cairo conference general assembly invited participation of NGOs

Bill Clinton reversed the Mexico City Policy → served as a signal to feminist groups that the political climate has changed

1975-85: UN Decade for the Advancement of Women

- Increase in research on women's living conditions
- Criticism of previous developmental projects discarding women

Rio Conference (United Nations Conference on Environment and Development (UNCED))- 1992:

- Research and studies linking population variables to environmental ones
- Aim to bridge the gap between environmentalists, feminists

FEMINIST CAMPAIGNS

Women's Liberation Movement

1993: Population policies by the International Women's Health Coalition

2 new chapters:

- **Gender equality and the empowerment of women**
- **Reproductive rights, reproductive health and family planning**

POLITICAL CLIMATE

Clinton-Gore team during elections

- Both were vocal about global population growth rate concerns
- Liberal view on abortion and family planning

Reinstated funding for UNFPA, IPPF

Timothy With: Counselor in-charge of population matters in Dept. of State

State: global security, economic issues, sustainable development

USAID: family planning, maternal and reproductive health

US Govt., Holy Sea, Women's Movement

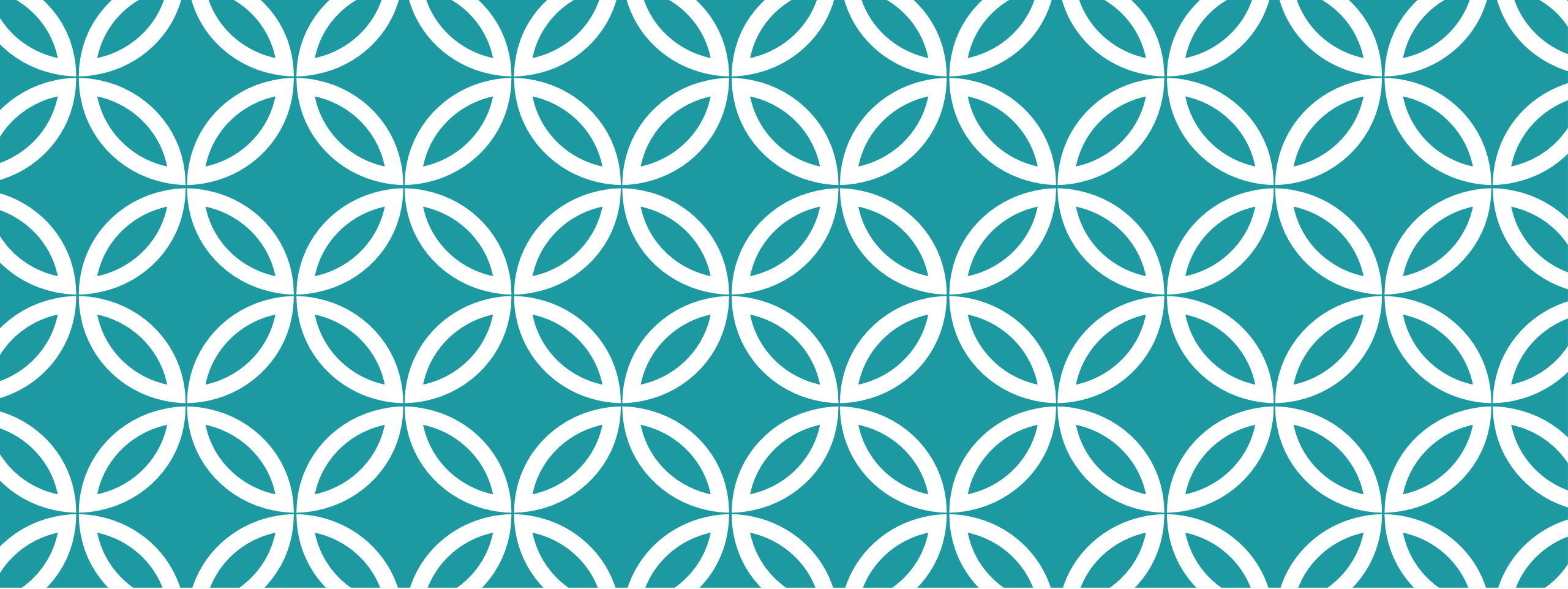
CONCLUSION

Success of conference measured in terms of new symbols/concepts and several 'firsts':

- **First mention of legal and safe abortion**
- **Women's interests considered**
- **Level of NGO participation**

Cairo recommendations:

- improvements in health, education, social, economic, political welfare.
- Long term aim to reduce fertility



A POPULATION OUT OF
CONTROL? MYTHS ABOUT
MUSLIM FERTILITY IN
CONTEMPORARY INDIA

Jeffery and Jeffery
(2002)

INTRODUCT

A Population Out of Control? Myths About Muslim Fertility in Contemporary India

Patricia Jeffery, Roger Jeffery

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- Communalization of politics
 - Politicization of population
 - Rhetoric by the Hindu Right
- "The communalization of politics in contemporary India has been buttressed by images of vulnerable Hindus (supposedly) doomed to be outnumbered. **Assertions about Muslim fertility and about Muslim men's right to marry polygamously have been key to the Hindu Right's propaganda. Such communalist population myths are "short on facts, short on a proper understanding of the demographic situation-- and short on honesty"** (Krishnakumar, 1991, p. 94)
 - Dangerous propaganda in **the victimization of minority groups that contributes to creating the very differences that are used to justify demonizing Muslims.**

DEMOGRAPHERS VERSUS HINDU RIGHT

Demographers have addressed Hindu-Muslim fertility through themes **like regional differences, variations in socioeconomic position, and occupation** rather than religion

Hindu Right echoed allegations repeatedly made in Hindu nationalist discourse since the early 20th century that Muslims constitute a threat to the “Hindu community” creating an “**Other**”

ISLAM AND FAMILY PLANNING

Muslim fertility is significantly higher than that of Hindus (??). Since the early 1970s, **Hindu Right propaganda** has also vigorously insisted that Indian *Muslims are anti-national because of their supposed refusal to adopt “modern” contraception*

The view that Islamic doctrine opposes family planning and that Muslims docilely respond to their religious leaders' propaganda draws on (and feeds into) a much wider pattern of portraying Muslims as dominated by Islam

Different understandings even among Muslims

In several other Muslim-majority countries, large numbers of men and women have undergone sterilization, including Iran where it was banned after the Revolution of 1979 but made available again after 1989

SOCIAL FACTORS

Victimize muslim women and perpetuate their backwardness

Contemporary demographers' preoccupations with the links between women's autonomy, 'girls' schooling, and fertility

Muslim men are stereotyped as more sexually active than Hindu men and as wanting more sexual partners and more children from them.

Muslim women are cast as victims of Muslim men's (supposedly) excessive sexual appetites. Yet the proportion of Muslim men with more than one wife is small and about the same as or less than among Hindus

Polygamy- not sufficient data to make large demographic inferences

RHETORIC THAT PREVAILED(S)

Polygamy “turns Muslim women into sexual objects and breeders,” that for every five Hindu children born, Muslims have 50 and that Hindus would become a minority in India within 25 years

Muslim men allegedly have four wives and say, “*ham paanch, hamaare pachis*”

REALITY: NFHS-5 DATA ON POLYGAMY

National Family Health Survey (NFHS)-5, showed that polygynous marriage rate:

Muslims (1.9%),

Other religious communities (1.6%)

Hindus (1.3%).

Polygamy in India: Insights and debates surrounding Uniform Civil Code

As the possibility of implementing a UCC is considered, it becomes crucial to delve into the data regarding polygamy and the various ways in which marriages are conducted.

<https://www.indiatoday.in/law/story/uniform-civil-code-polygamy-controversy-data-muslims-tribes-2400370-2023-06-30>

WHAT DEMOGRAPHERS SAY?

Demography studies done along socio- economic lines

- ▢ Examine the impact of polygamy, along with, for instance, **age of marriage, length of breast-feeding, length of postpartum sexual abstinence, or other periods of sexual abstinence.**
- ▢ But they treat these as proximate variables affecting fertility ***without making any assumptions about people's intentions.***
- ▢ Demographers' elaborate statistical analyses on large-scale data sets are not readily sensitive to local-level variations

Macro-level:

Indian demographic statistics indicate that Muslim fertility rates are somewhat higher than those among Hindus

Muslims make up a (slowly) growing share of the Indian population (up from 10.5% in 1951 to 12.5% in 1991).

WHY DO WE NEED TO EDUCATE PEOPLE ABOUT THE POPULATION?

Shivakumar Jolad
and Trishala S, Jun
23, 2023

False notions about
the population have
created fear,
prejudice, and
communal hatred.

The perpetuation of fears about the Hindu-Muslim population divide, which has resulted in violence and discrimination against Muslim communities in India. RSS chief Mohan Bhagwat routinely invokes the need for “religion-based population control” policies. Yogi Adityanath claims that the Muslim population increased multifold because they got special rights and privileges, which has created fear that Muslims will outstrip the Hindu population.

The changes in the religious makeup of the population have been modest since 1951. Data shows these are a figment of imagination of prejudiced minds. The Hindu population grew from 30 crores to a billion from 1951 to 2011, whereas the Muslim population grew from 3.5 cr to 17.2 crores at the same time. Much of the growth differential happened during the early differential in growth rate, which is now converging faster. In a span of 60 years (1951–2011), the percentage of Muslims has modestly increased from about 10% to 14%. The 2021 projection of the Muslim Population is about 15%. Pew Center projects that by 2050 (close to when India might achieve population stabilization), Muslims may see a modest rise of 18%- still less than 40% of Hindus (77%) and far below the fears of Muslim overshooting the Hindu population.

<https://shiva-jolad-46811.medium.com/why-do-we-need-to-educate-people-about-the-population-5ca41e1e43fc>

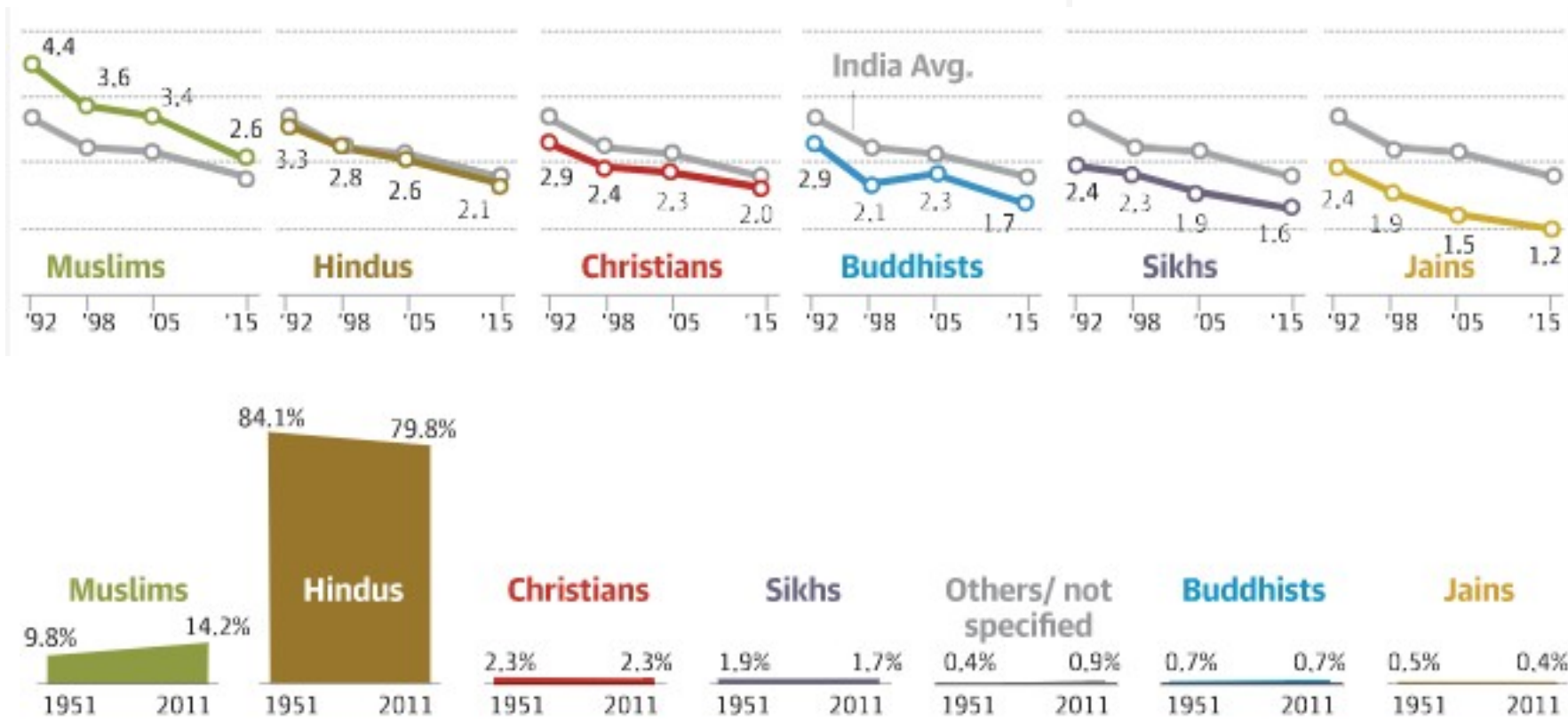
Data | Fertility rates decline across religions in India, sharpest drop recorded among Muslims



The Hindu Data Team

SEPTEMBER 23, 2021 16:11 IST

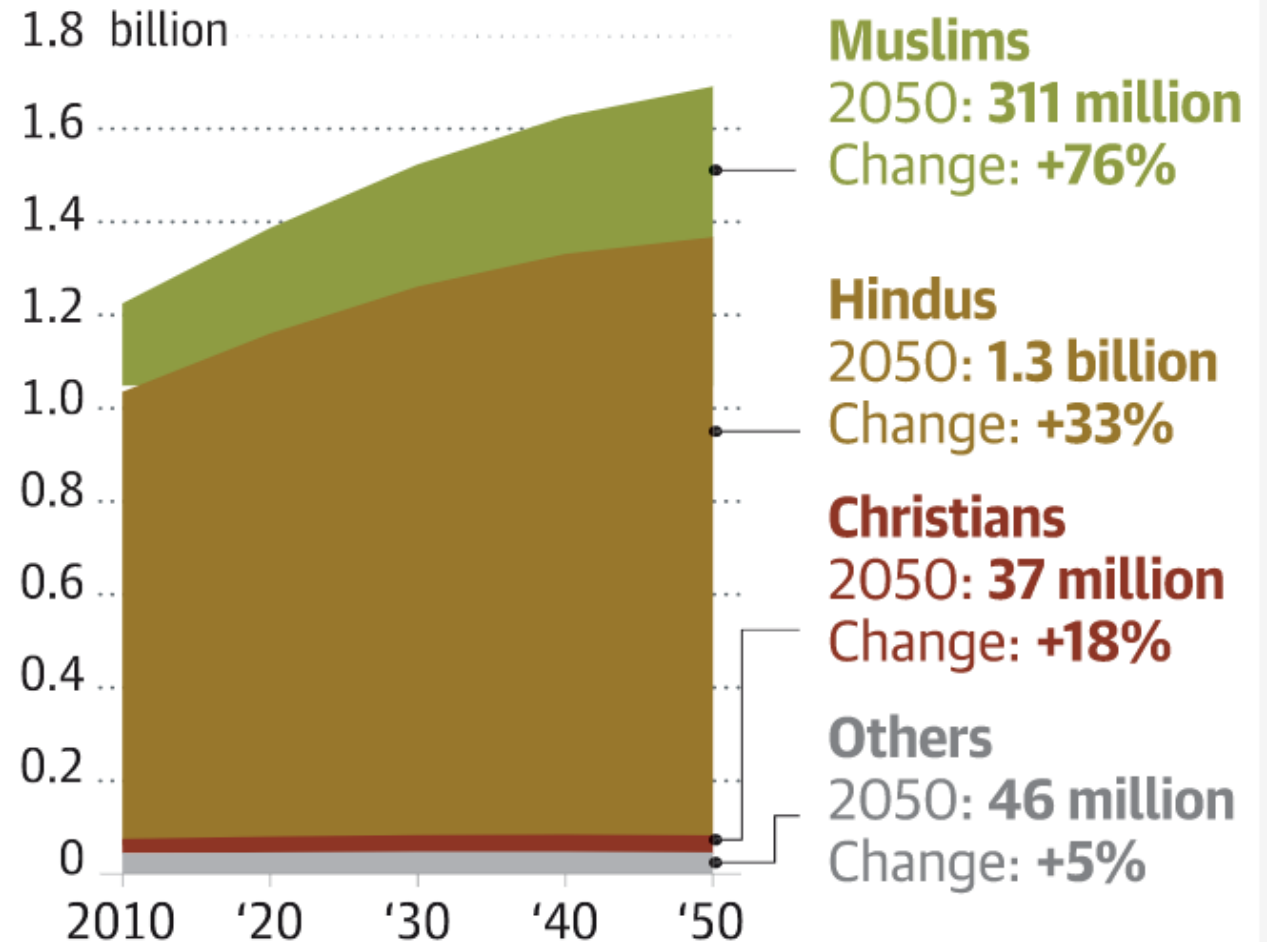
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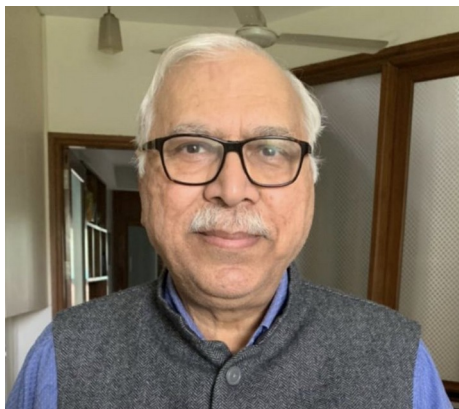
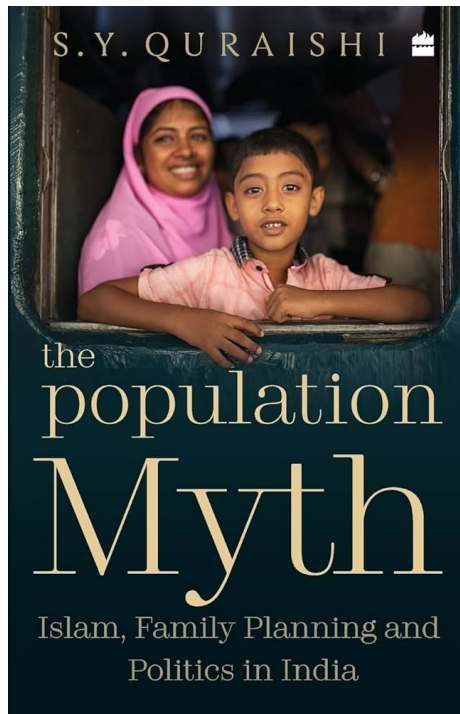
<https://www.thehindu.com/data/data-fertility-rates-decline-across-religions-in-india-sharpest-drop-recorded-among-muslims/article36625885.ece>

STABLE EVEN AT 2050

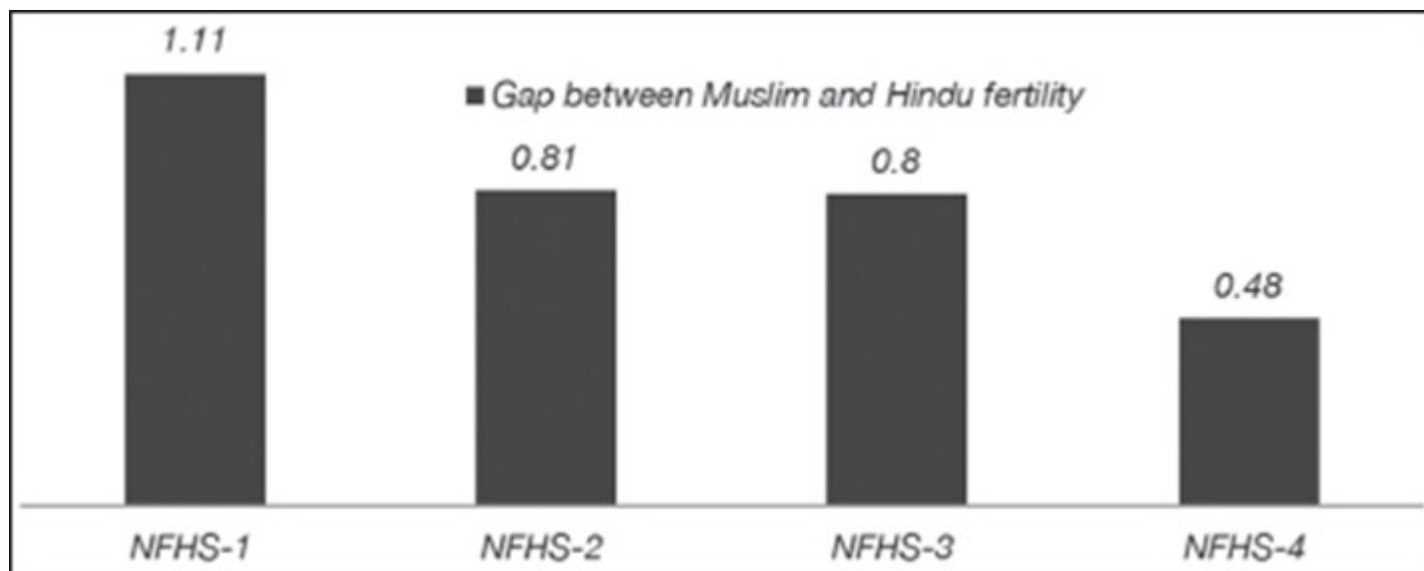
- According to Pew Research Center's demographic projections for 2050,
- Hindus will form 77% of India's total population (a 2% point decrease from 2020)
- and the share of Muslims will grow by 3% points to 18% in 2050).



In UP, the Population policy had a communal angle- with Yogi Adityanath stating that it is an effort to maintain population balance among various communities. Assam Chief Minister Himanta Biswa Sarma, who believes that “population is the root cause of every social menace” similarly justified his stance and wants the “immigrant Muslim community adopts decent family planning norms” to “solve numerous social ills in Assam. The fertility rates in these states tell a different story. UP’s Total Fertility Rate (TFR), according to NFHS rounds 1 to 5, dropped from 4.8 in 1992–93, rapidly dropped to 2.7 in 2015–16, and now recently stands at 2.4- approaching replacement level. The Muslim TFR (2.7) in 2020–22 is just 0.3 is higher than the average TFR of UP, and is at the same level as in 2015–16. The TFR (NFHS-5) of Assam is already well below replacement level (1.9) a sharp drop from 3.5 in 1992–93. The Muslim fertility is 2.4, with a Total wanted fertility rate of 1.8, showing a gap of 0.6 — primarily due to unmet needs of fertility rather than the true intentions.



In the book — “The Population Myth: Islam, Family Planning and Politics in India”, SY Quraishi (former Chief Election Commissioner of India), debunks the myths about Muslim fertility and population using data from Census and Family Health surveys. He shows that although the Muslim population percentage has been increasing, the decadal population growth rate has been decreasing faster than Hindus. Muslim decadal population growth rate decreased from 30.9% in 1951–61 to 24.6%, the corresponding change for Hindu growth rate was 20.7% to 18.5%. The fertility gap between Muslims and Hindus decreased from 1.11 in 1992–93 (NFHS-1) to 0.48 in 2015–16 (NFHS-4). And recently it has dropped further to 0.42 in 2029–21 (NFHS-5). Quraishi notes that the “TFRs of Hindus in Bihar and Rajasthan far exceed that of Muslims in Kerala or Tamil Nadu”. Fertility is more linked to women’s education level, empowerment, and access to family planning methods, rather than to religion. The fear of the Muslim population overtaking it has no empirical basis.



NFHS-5

Fertility
gap = 0.37
Hindu
1.94 ,
Muslim = 2.
3

Year	Muslim population (in 000s)	Absolute increase in Muslim Population (in 000s)	% Increase in Muslim Population	Hindu population (in 000s)	Absolute increase in Hindu population (in 000s)	% Increase in Hindu population	Difference in increase in PP
1951	35,856			3,03,676			
1961	46,941	11,085	30.9%	3,66,528	3,66,528	20.7%	10.2%
1971	61,418	14,477	30.8%	4,53,292	86,764	23.7%	7.2%
1981*	80,286	18,868	30.7%	5,62,389	1,09,097	24.1%	6.7%
1991**	1,06,715	26,429	32.9%	6,90,060	1,27,671	22.7%	10.2%
2001	1,38,188	31,473	29.5%	8,27,579	1,37,519	19.9%	9.6%
2011	1,72,245	34,057	24.6%	9,80,378	1,52,799	18.5%	6.2%

<https://scroll.in/latest/1023559/muslims-fertility-rate-sees-sharpest-decline-over-two-decades->

Coercive population control policies

Coercive policies to control the population will always backfire. During the 1975–77 Emergency period in India, a repressive family planning program of forced sterilizations (Sanjay Gandhi's Five-point program) was implemented, leading to an estimated 6.2 million men being sterilized in 1976 alone, with over 2,000 deaths due to botched operations. The Emergency saw targeted coercion against marginalized groups, including Muslims and tribals. Amartya Sen, in his article “Population Policy: Authoritarianism Versus Cooperation” (1994), discusses the dichotomy between “an authoritarian or a cooperative approach” to population control. Quoting Samuel Coleridge (1830), “What begins in fear usually ends in folly”, Sen argues for cooperative and persuasive methods can lower fertility rates more effectively than authoritarian methods. He cites that between 1979-1991, Kerala reduced fertility from 3 to 1.8 by promoting literacy among women, whereas China's with its draconian one-child policy reduced TFR only from 2.8 to 2.0.

India. The Cairo Conference of 1994, was indeed a significant event that addressed various important issues related to human rights, population, sexual and reproductive health, gender equality, women's rights, and sustainable development. Cairo's Program of Action, which was adopted by most countries including India. It emphasized the sexual and reproductive rights and reproductive health for fertility reduction, as opposed to coercive demographic target driven policies. Since then, India has transitioned towards a voluntary and rights-based approach, but some states have implemented "two-child policies" that limit options and civil rights for those with more than two children. Violations can result in disqualification from government jobs, welfare schemes, and the right to contest local elections, raising concerns about coercion and reproductive rights. It is women who primarily bear the physical and emotional trauma of two child policies. Sweta Dash notes that these policies can have unintended consequences such as unsafe abortions, coercive targeted sterilizations, a spike in female foeticides, and reduced access to affordable reproductive and maternal healthcare.

Myths about Migration

India is the origin of worlds largest number of International migrants, estimated to be around 18 million. It receives very few immigrants, accounting for less than 0.4 per cent of its 1.42 billion population. The net migration rate is negative at -0.2 migrants per thousand population. Yet, there are fears about massive illegal migration (especially from Bangladesh, Pakistan, and Myanmar), particularly in the North East region of India. India receives the lowest number of international migrants (in proportion to population), both legal and illegal, compared to other countries. Politicians make wild claims about illegal immigrants. Former Minister of State for Home Kiren Rijiju made a claim in 2016, that there are 20 million illegal immigrants in India. The data from Census and other sources hardly support this. Census 2011 shows 2.3 million migrants from Bangladesh. Most of them migrated before 1991 numbering 1.76 million. Bangladeshi don't prefer moving to India anymore. Only about 84,000 emigrated between 2006–2011, according to Census 2011 data. Only 40,000 Rohingya Muslims from Myanmar are estimated to have migrated to India. Yet, India built a narrative about excessive illegal immigration from these countries and used it to justify Citizenship Amendment Act (CAA)-2019, allowing only non-Muslim persecuted minorities to seek citizenship in India.

While the Indian Constitution grants fundamental freedoms of movement and choice of livelihood across the country, migrants continue to face harassment and exclusion from nativist minds in the cities. Despite migrants being the backbone of city building, as seen in cities like Mumbai or Bangalore where they enrich the cultural and linguistic diversity, they are often met with hostility and hatred from both locals and politicians. This divisive atmosphere contributes to the marginalization and insecurity experienced by migrants in the region. During the 1960–80s, Maharashtra created fears among migrants and fostered nativism. Earlier targets were largely Tamilians and other south Indians (clubbed into 'Madrasi' group) migrants to Mumbai, later it was North Indian migrants mainly from UP and Bihar who were attacked and harassed. Anti-migrant attacks spread to other cities such as Bangalore. In 2012, thousands of panic-stricken people from the northeastern states living in Bangalore boarded trains to Guwahati due to rumours of violence targeting them. The cosmopolitan Bangalore, known for its linguistic diversity (107 languages are spoken here), created an atmosphere of fear and mistrust among migrants from the northeast.



UNMET NEED FOR FAMILY PLANNING IN DEVELOPING COUNTRIES AND IMPLICATIONS FOR POPULATION POLICY

JOHN B. CASTERLINE & STEVEN W. SINDING

BACKGROUND

- ❑ **The condition of wanting to avoid or postpone childbearing but not using any method of contraception.**
- ❑ Purpose: Utility of unmet need as a way to formulate population policies
- ❑ **1960's: KAP - Gap (Knowledge, attitude, practice)**
- ❑ 1970's: World fertility survey (WFS)
- ❑ **Contraceptive Prevalence Surveys (CPS) project - broadened the definition of unmet need.**

VALIDITY OF CONCEPT

- ❑ The fundamental challenge to the concept of unmet need is the assertion that it does not refer to a valid behavioral phenomenon. However, found to be in the eyes of the analyst who infer this from the survey.
- ❑ **The most direct evidence that unmet need is real is the high incidence of pregnancies that are reported as unintended.**
- ❑ **Also, qualitative surveys shows that men and women feel frustrated at their inability to adopt behaviours that would effectively prevent unintended pregnancies.**
- ❑ Those who perceive themselves as infecund will not use contraception due to additional costs

CRITIC OF UNMET NEEDS

- Two main criticisms to the validity of unmet needs: concept is illogical and the concept is patronizing.
- However, **it was found that preferences to avoid pregnancy are stymied by various constraints and obstacles, most notably fear of health side effects and social opposition.**
- **Study in Nepal found that women and men do not use a method despite a clear desire to avoid pregnancy**

UNMET NEED AND FERTILITY TRANSITION

- Does unmet need have any observable association with trends over time in contraceptive prevalence and fertility?
- **Adoption of contraception should be one of the common resolutions**
- Pritchett claims that reductions in unwanted fertility must contribute very little to fertility decline.
- However, **Bongaarts argues that reduction in unwanted fertility can only occur if fertility rates fall among the growing subset of women who want to stop childbearing.**
- Substantial increases in contraceptive prevalence can be achieved in the absence of changes in the demand for children, through the satisfaction of already-existing demand for fertility

UNMET NEED AND DEMAND FOR FAMILY PLANNING

- Devising unmet need is perceived as a potential avenue to estimate latent demand for family planning
- Equating unmet need to demand for family planning has been common
- This latent demand might be converted into contraceptive use
- Intention to use - another good proxy for contraceptive behaviour due to high correlation

BROADENING THE DEFINITION

- ❑ Current concept deemed narrow
 - No inclusion of unmarried women
 - Qualitative and quantitative aspects must be taken : degree of dissatisfaction with current contraceptive method and quality of services provided
- ❑ Critical rationale of definition - unintended pregnancies are undesirable
- ❑ Male fertility preferences - must be partner specific
- ❑ Concept of couple unmet need - must record couple discordance
- ❑ Must exclude women using “traditional” contraceptive methods from the figures of those using contraception as these are pretty ineffective – unmet need in Vietnam increased from 14% to 36% once this was

UNMET NEED AND PROGRAMMATIC ACTION

- Assumption – unmet need can be satisfied through programmatic interventions
- Non use of contraception can be due to “access causes” or “non-access causes” (socio-cultural barriers)
- Latter has been cited as the main reason; programmatic interventions must be aimed at reducing such barriers
- Other barriers identified – lack of knowledge, social barriers and health concerns
 - Social barriers – most women convinced that husband’s approval must be there to practice contraception, husbands concerned about societal approval
 - Health concerns also extremely powerful – women don’t want to deal with financial and emotional costs of side-effects, potential relationship problems, physical discomfort
 - Health interventions can include counselling of women, periodic health check-ups for adopters of contraception

UNMET NEED AND POPULATION POLICY

- Mistake of equating population policies with only family planning
- Focus should also be on female schooling, reducing infant and child mortality
- Tackling unmet need would ensure tackling causes of unmet need
- Thus, appropriate interventions can be introduced to reduce social barriers to and specific causes of unmet need